

The Skilled Conversation



AMENDMENT

In December 2024, Section 9, Good Information and Advice and Prevention was updated to include information about carers leave.

[December 16, 2024](#)

1. Using this Procedure

This procedure should be used by anyone that has been allocated or asked to carry out any process that establishes the eligible needs of a person or carer under the Care Act, including any formal assessment or reassessment of need.

2. A Strengths Based Approach

Wherever possible, every conversation with a person/carers should be from a strengths perspective. This means that before you talk about service solutions to the presenting issue you must support the person/carers to explore whether there is:

- a. Anything within their own power that they can do to help themselves; or
- b. Anything within the power of their family, friends or community that they can use to help themselves.

A strengths based approach is empowering for the person/carer and gives them more control over their situation and how best to resolve any issues in the best way for them. The end result may still be that the Local Authority intervenes with support, but this decision will have been reached knowing that it is the most proportionate response available.

Adopting a strengths based approach involves:

- a. Taking a holistic view of the person's/carer's needs in the context of their wider support network;
- b. Helping the person/carer to understand their strengths and capabilities within the context of their situation;
- c. Helping the person/carer to understand and explore the support available to them in the community;
- d. Helping the person/carer to understand and explore the support available to them through other networks or services (e.g. health);
- e. Exploring some of the less intrusive/intensive ways the Local Authority may be able to help (such as through prevention services or signposting).

SCIE have produced clear and practical guidance around how to use a strengths based approach in practice. See: [Care Act guidance on Strengths-based approaches](#). **Note:** SCIE requires a login to access resources, but any social care practitioner can create one quickly and easily.

3. Carers (Adults Needs Assessments)

Under the Care Act carers have to be a part of the cared for person's assessment, even if the cared for person does not want them to be. This is because the Local Authority needs to:

- a. Understand the person's needs, how they are currently being met and whether this is appropriate and sustainable; and
- b. Identify and support carers.

When a person has not consented to the carer being a part of the assessment you should:

- a. Advise the person that you have a duty to involve the carer;
- b. Explain the benefits of the carer being involved; and
- c. Agree the most appropriate way to involve the carer (for example a separate meeting with the carer).

If the person has requested particular information not relating to needs is withheld from the carer, and they have capacity to do so normal confidentiality rules apply *unless* doing so would put the person (or another vulnerable adult or child) at risk of abuse or neglect.

Example:

Pritesh is a carer for his brother Ash. Ash is happy for Pritesh to be involved in the assessment, but asks the social care practitioner not to discuss difficulties he is currently having in his relationship with his girlfriend while Pritesh is present, as he feels this is a private matter and bears no impact on his care needs.

4. A Whole Family Approach

4.1 What the Care Act says

Under the Care Act it is important that the person's/carer's needs are considered within the context of their family network of support and not in isolation. This is what is meant by a whole family approach.

Taking a whole family approach enables the Local Authority to:

1. Understand the impact of the person's Care and Support needs on all family members, not just those that may be instantly recognisable as carers; and
2. Identify and support carers (both these things are duties under the Care Act).

Taking a whole family approach also builds the resilience of families and increases the likelihood that a non-service led self-sustainable method of meeting needs will be identified.

Need to know

The direct involvement of family member's in a needs assessment (other than family members that are carers under the Care Act) is subject to consent being provided by the person/carer *unless* a person with Care and Support needs lacks capacity to do so and a best interest decision is made to this effect.

When taking a whole family approach you should consider involving the following people in the assessment:

- a. Adult's living in the same household as the person/carer;
- b. Children living in the same household as the person/carer;
- c. Adults living elsewhere but who form part of the person's/carer's support network;
- d. Children living elsewhere but who form part of the person's/carer's support network.

Specifically you need to understand:

- a. The impact of the person's needs on family members (both adult and children);
- b. Whether the role that other adults or children in the family have constitutes a caring role under either the Care Act 2014 or the Children and Families Act 2014;
- c. Whether the support currently being provided to adults and children to manage the impact is appropriate (or whether a carer's/young carer's assessment should be offered); and
- d. Whether the support being provided is sustainable.

Where there is an appearance of need in a carer/another carer you have a duty to offer an assessment.

4.2 Adult family members

Any conversations with adult family members must give regard to the duty to provide good information and advice about adult Care and Support. This could be in relation to the needs of the person being assessed (or being cared for by the carer), in relation to their own apparent needs or in relation to another adult that they support or are concerned about.

You must also consider whether any prevention service would be of benefit to any adult family member you speak to, either to support them as a carer for the person or in their own right. Where they appear to have a need for a prevention service themselves you should support them to access this service.

Example:

Joy has a sister called Mary. As part of Joy's assessment you consult with Mary who is helping Joy to do her shopping each week. Mary tells you that she wishes she lived in a bungalow like Joy, because she is starting to find using the stairs in her own home difficult due to reduced mobility. You offer advice to Mary about the possible benefit of an assessment from an Occupational Therapist. Mary considers this and feels it would be of benefit to her, so you support Mary to make the referral for an assessment.

Where you take action to support another adult in the person's/carer's family via a whole family approach you should make sure that you make appropriate and proportionate recordings for the action you have taken or the information and advice you have provided.

4.3 Child family members

The Duty to safeguard children

Children are protected under different legislation to adults. Whilst you are not expected to understand children's legislation in your role you are expected to know your statutory responsibilities in relation to protecting children so that you can respond appropriately.

Things that you must consider in relation to all children living in the household:

- a. Whether the child is taking part in any caring activities that may be inappropriate for their age, gender or that cause them physical difficulty or emotional distress;
- b. Whether the adult with Care and Support needs presents with behaviours that place the child at risk of harm or abuse;
- c. Whether the child is at risk of neglect because the adult with care and support needs has caring responsibilities towards them that they cannot adequately meet as a result of their own needs;

d. Whether the child is at risk of neglect because a carer has caring responsibilities towards them that they cannot adequately meet as a result of caring for an adult;

e. Whether the home environment or any caring role is having a negative impact on the child's well-being, education or development.

If you are concerned about the safety of a child, or that they are at risk of abuse and neglect you **must** take action to protect them.

If you are concerned that a child is in imminent danger from abuse or neglect, or that a criminal act has taken place you should contact the police by dialling 999.

Responsibilities to young carers

The legal definition of a young carer under the Children and Families Act is:

"a person under 18 who provides or intends to provide care for another person (of any age, except where that care is provided for payment, pursuant to a contract or as voluntary work)".

There is a duty to meet the needs of young carers under the Children and Families Act in the same way as there is a duty to meet the needs of adult carers under the Care Act. This means that if you become aware that a child or young person under the age of 18 is providing care you must take steps to ensure they are appropriately supported.

The first thing you should do is liaise with children's services to establish whether the child or young person is already being supported as a young carer.

If support is in place you should provide children's services with any new information you may have about the impact of the adult's needs upon the child, but you will not need to provide any further support to the child.

If the young carer is not already being supported by children's services you should discuss and agree with your line manager and children's services who is best placed to assess them.

Option 1

If the young carer is not approaching the age of transition any assessment and subsequent support they receive will be provided under the Children Act 1989. The practitioner carrying out a young carer's assessment under the Children Act must be suitably skilled, knowledgeable and competent to do so.

Option 2

If the young carer is approaching the age of transition (or is at a time of their life when it would be of significant benefit for transition to begin) a young carer's assessment under the Children and Families Act or Care Act may be more appropriate. This will involve the development of a transition plan that will support the young carer through the transition to adult Care and Support (when they become 18).

There is a legal duty to ensure that transition planning and the whole transition experience is positive for the young carer. To this end an element of joint work will likely be required to ensure the young carer receives the support they need both now and into adulthood.

For further information about young carer's transition assessments, see: [Transition to Adult Care and Support](#).

5. Talking about Individual Wellbeing

5.1 Why you must talk about Wellbeing

Wellbeing is the single most important concept of the Care Act. The duty to promote individual wellbeing applies at all times; in every single process, conversation or decision that is made and you **must** be able to demonstrate that you have done so.

It is vital that you understand your duties in relation to promoting individual Wellbeing. See: [Promoting Individual Wellbeing](#).

5.2 What you must establish about Wellbeing

In order to promote individual Wellbeing you must first understand what Wellbeing means to the person/carers. The Care Act sets out 9 domains of Wellbeing that you must consider. See: [Wellbeing Domains](#).

As part of any process to establish needs you **must** understand:

- a. Which areas of Wellbeing are most important to the person/carers at that moment in time;
- b. Which areas of Wellbeing are least important at that time;

- c. Whether there are other areas of the person's/carer's life important to them but not listed as a domain (the domains under the Act are not definitive as Wellbeing is personal);
- d. Which areas of Wellbeing are causing the person/carer concern or worry;
- e. What impact any concern or worry is having (or could have) across the Wellbeing domains (is there a destabilising effect?); and
- f. How the person/carer thinks any Care and Support/Support needs interact and impact on Wellbeing.

It is important to remember that under the Care Act the Wellbeing domains are all as important as each other. Any hierarchy can only be determined or described by the person/carer whose Wellbeing it is.

5.3 Important rules when talking about Wellbeing

- a. A sense of Wellbeing is extremely personal and will be different for every person/carer;
- b. Never make assumptions about what is important and what is not important to a person/carer;
- c. The person/carer is the expert in relation to their own Wellbeing;
- d. If a person/carer tells you they have no worries in a particular area this does not mean the area is not important to them; and
- e. Conversations about Wellbeing should be 'genuine' person centred conversations, not process led.

5.4 Preparing to talk about Wellbeing

Remember

See the [tri.x Resources](#) to access additional practice guidance that can support the processes of establishing needs, Care and Support Planning and review when a person has specific or complex needs.

Often the key to an effective conversation about Wellbeing is preparation and you should take whatever steps are available to ensure that a person/carer is as prepared as possible for the conversation:

- a. Provide information that will support the person/carer to understand the concept of Wellbeing and why it is important;
- b. Offer advice about the things that a person/carer may want to think about before having a conversation about Wellbeing;
- c. Make sure that anybody who will be supporting the person/carer understands Wellbeing and is confident and appropriate to support the person to prepare for and have the conversation; and
- d. Consider whether any tools may be helpful to support the person/carer to think about Wellbeing.

Need to know

Remember that using a tool to support the process of establishing needs or formal assessment can be useful for all involved.

5.5 Talking about Wellbeing

A conversation about Wellbeing can be a very difficult conversation for a person/carer to have. Not everyone will feel happy, confident or able to share with you how they are feeling about the different areas of their life. For many people, this will be the first time they have been asked or given an opportunity to think about their own Wellbeing in this way, and they may also be doing so whilst adapting to significant change in their life.

Any questions that are used to support a person/carer to think about Wellbeing must:

- a. Be proportionate to the level of information required;
- b. Be appropriate, taking into account the person's/carer's specific needs around communication and their specific circumstances;
- c. Be realistic in respect of mental capacity and a person's/carer's ability to be able to answer the question; and
- d. Be asked in a manner that is accessible to the person/carer.

Powerful questions

A powerful question is a specific type of open question that:

- 1. Encourages a person/carer to reflect;
- 2. Is thought-provoking;
- 3. Supports an exploration of options; and
- 4. Helps the person/carer to gain a greater insight into their situation.

Powerful questions should be framed in a positive way to promote engagement of the person/carer and promote a strengths based approach.

Powerful Question	Open Question (not powerful)
Why do you think that means so much to you?	What do you think that for?
What works well about the support you have?	Who does that for you?
Why do you think that didn't quite go as expected?	Why didn't that work out?
What made you decide to take that approach?	Why did you do that?

Appreciative Enquiry

An appreciative enquiry is a conversation that is led by the person/carer and focuses on times of personal strength. It supports them to recognise that they do still have those strengths and abilities and to think about how they can apply them to their current situation.

The listener should invite the person/carer to:

- a. Talk about a time or times when something has been working well in their life;
- b. Explore what it was that worked well and supported them at that time;
- c. Think about how that experience could support them now in making a plan for the future.

Some key questions to support an appreciative enquiry approach include:

- a. Tell me about a time when things were going well for you?
- b. What did you learn about your strengths at that time?
- c. If you had a magic wand what would the future look like?
- d. What is it that you value most in your life now?
- e. What small changes would make the most difference?

The use of tools

A tool can be helpful to shape and focus a conversation about Wellbeing, making sure that you consider everything that needs to be considered from a statutory perspective. Tools are also useful because they:

- a. Can be completed with the person/carer as part of any conversation you have with them; or
- b. The person/carer can complete them in their own time as part of their preparation for the conversation.

See: [Tools and Practice Guidance to Establish Needs](#) to access the range of tools that are available and practice guidance around the use of tools.

5.6 Wellbeing and Capacity

Some people with Care and Support needs will lack capacity to have a conversation or communicate how they feel about their own Wellbeing at a particular moment in time (verbally or through another means). For example, they may be too unwell to do so or have a significant learning disability.

Where this is the case the duty to ensure their involvement still applies.

There are a range of ways that you can ensure the involvement of a person who lacks capacity, including but not limited to:

- a. An appropriate other person or independent advocate to support the person to engage and ensure that they are represented;
- b. Spending time with the person can show you what they enjoy about life and what may be most important to them (this could be a person, a place or something they do with their time);
- c. Consulting with a range of people who know the person before reaching a judgement about Wellbeing. Speaking to a family member, a health professional, a paid carer, a college tutor and a day services manager will give a much better picture of what appears to matter most to the person than relying on the views of one person;
- d. Use other available evidence to support you to understand Wellbeing (for example ABC charts and other records that show behaviour changes clearly linked to an event, person or place).

All information gathering and sharing should be carried out with regard to the Caldicott Principles, Data Protection legislation and local information sharing policies.

5.7 Exploring and Managing different views about Wellbeing

Because Wellbeing is extremely personal different people are likely to have different perspectives about the same domains. The person/carer themselves is more likely to have a subjective view about their Wellbeing which may be dominated by worries or concerns they have about their presenting situation (for example the potential impact of a serious health diagnosis), whereas someone who is emotionally removed from may see things more objectively.

It is important that you recognise when a person's/carer's views about their Wellbeing may not be holistic and take steps to try and support them to gain insight or broaden their thinking. One way of doing this is to seek the views of family members and others involved in their life (with the person's/carer's consent or in the best interests of a person with Care and Support needs if they lack capacity). This supports a whole family approach to assessment but can also lead to a shared understanding and solution from within informal networks of support.

Even with the involvement and perspective of others certain factors effecting Wellbeing may still be difficult for people to recognise, acknowledge or explore. If it is your view that these things are having an impact on Wellbeing then you must raise them for the purposes of:

- a. Supporting the person/carer to gain insight into their situation;
- b. Developing the person's/carer's resilience through talking about difficult situations and exploring options to find a solution;
- c. Providing information and advice;
- d. Preventing, reducing or delaying the need for Care and Support/Support.

Example 1:

John has recently been diagnosed with cancer. However, he is declining to acknowledge his diagnosis and talk about the future. Instead he focuses his thoughts on his work and tells his social care practitioner that he has no worries about his health when having a conversation about Wellbeing. Even though it is challenging for the practitioner they talk to John about his diagnosis because by ignoring it the duty to prevent, reduce and delay needs cannot be met.

Example 2:

George tells his social care practitioner that his finances are causing him the most concern but blames his current situation on a recent reduction in his welfare benefits. His social care practitioner knows from speaking to George's carer that he has become involved in an online gambling site, and that this is the true cause of his recent deprivation. The potential impact on George's Wellbeing if he does not address his gambling issue is vast, and the social care practitioner has no option but to raise the issue so that George gains insight into the consequences and she is able to provide him with the right information and advice to move forward.

You may also find the tool [Six Tips to Help You Have Difficult Conversations](#) useful.

You should also make effective use of supervision to explore and develop skills that will support you to have difficult and sensitive conversations with people in a positive way.

5.8 Talking about Wellbeing in a Crisis

A person's/carer's wellbeing is always affected by what is happening in their life at that time. It changes as their situation changes, and sometimes the changes in Wellbeing that take place can be quite dramatic. It is therefore important to understand a person's/carer's Wellbeing in the context of their current situation, but to monitor and review Wellbeing as things change.

5.9 Promoting Wellbeing

When you have understood a person's/carer's individual Wellbeing you have a duty to promote it. To do this you must understand:

- a. The things that they want to achieve;
- b. The things they want to change; and
- c. The things that they want to stay the same.

These things are known as outcomes. In order to promote Wellbeing you should explore with the person/carer during the assessment the steps that they can take to achieve their outcomes, as the two things are intrinsically linked.

NHS Choices have an online resource: [5 steps to mental wellbeing](#).

If, following assessment the Local Authority meets any eligible needs that the person/carer has you will need to explore not only what steps the person/carer can take to achieve their outcomes, but also how the support/services provided can help them do so too.

Need to know

Outcomes should reflect the things that the person/carer wants to achieve and not what other people want to happen.

5.10 Balancing the Wellbeing of others against the Wellbeing of the person/carer

You have a duty to promote individual Wellbeing but this is not a duty at the detriment of others. The Wellbeing of others must be taken into account

and may determine the action that you then do or do not take to promote a person's/carer's Wellbeing.

For example, if promoting the person's/carer's Wellbeing will put a vulnerable adult or a child at risk then you must not do so unless risks can be mitigated. The duty to protect people from abuse or neglect overrides the duty to promote individual Wellbeing.

6. Talking about Needs

6.1 Why you must talk about needs

The Local Authority has a duty to meet eligible needs, and it cannot fulfil this duty unless it establishes whether the needs that a person/carer appears to have are eligible.

A person's/carer's needs also often directly impact on their Wellbeing, which you have a duty to promote. Therefore understanding needs will also support you to fulfil the duty to promote Wellbeing.

6.2 The areas of need described in the Care Act

People with Care and Support needs

The Care Act sets out 10 areas of need that must be assessed whenever it appears that they exist:

- Manage and maintain nutrition;
- Maintain personal hygiene;
- Manage toilet need;
- Being appropriately clothed;
- Be able to make use of their home safely;
- Maintain a habitable home environment;

- Develop/maintain family and other personal relationships;
- Access/engage in work, training, education or volunteering;
- Make use of community services;
- Carry out caring responsibilities for a child.

Carers

The Care Act sets out 8 areas of need that must be assessed whenever it appears that they exist:

- Carry out caring responsibilities for a child;
- Provide Care and Support to other adults;
- Maintain a habitable home environment in their own home;
- Manage and maintain nutrition;
- Develop/maintain family and other personal relationships;
- Engage in work, training, education or volunteering;
- Make use of community services;
- Engage in recreational activities.

6.3 Proportionate and Appropriate conversations about need

Unlike a conversation about individual Wellbeing there is not a requirement to talk about all of the areas of need described in the Care Act, regardless of whether they appear to be present or not. Instead the conversation about needs must be proportionate and appropriate to each person's/carer's situation.

Example:

Simon is a single male. He has a mental health issue but has no physical disability. Unless further evidence was provided it would not be appropriate to ask Simon about needs in relation to managing the toilet and carrying out caring responsibilities for a child. Furthermore, when talking about areas (such as being appropriately clothed or maintaining personal hygiene) it would not be appropriate to ask Simon about his physical abilities to carry out tasks because, from the available evidence his needs relate to his mental health.

Example:

Jill is an older adult with Dementia. Jill finds it very difficult to remember the name of her daughters and carers and the physical symptoms of her condition have also deteriorated rapidly, to the point where she is no longer able to walk or eat without support. Due to the range of needs that Jill appears to have it would be proportionate and appropriate to talk about most of the areas in the Care Act, with the exception of work, training, education and volunteering and carrying out caring responsibilities for a child.

6.4 Preparing to talk about need

Need to know

Remember: see the [tri.x Resources](#) to access additional practice guidance that can support the processes of establishing needs, Care and Support Planning and review when a person has specific or complex needs.

Often the key to an effective conversation about need is preparation and you should take whatever steps are available to ensure that the person/carer is as prepared as possible for the conversation:

- a. Provide information that will support the person/carer to understand the areas of need described in the Care Act and why establishing needs is important;
- b. Offer advice about the things that a person/carer may want to think about before having a conversation about needs;

- c. Make sure that anybody who will be supporting the person/carer understands needs and is confident and appropriate to support the person/carer to prepare for and have the conversation; and
- d. Consider whether any tools may be helpful to support the person/carer to think about needs.

6.5 Talking about need

Need to know

Remember that using a tool to support the process of establishing needs or formal assessment can be useful for all involved.

A conversation about needs can be a very difficult conversation for a person/carer to have. Not everyone will feel happy, confident or able to talk about their needs. Some people may lack insight into their needs, particularly if they are still going through a period of adjustment to the need.

Any questions that are used to support a person/carer to think about needs must:

- a. Be proportionate to the level of information required;
- b. Be appropriate, taking into account the person's/carer's specific needs around communication and their specific circumstances;
- c. Be realistic in respect of mental capacity and the person's/carer's ability to be able to answer the question; and
- d. Be asked in a manner that is accessible to the person/carer.

Powerful questions

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- a. Encourages a person/carer to reflect;
- b. Is thought-provoking;
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d. Helps the person/carer to gain a greater insight into their situation.

Powerful questions should be framed in a positive way to promote engagement of the person/carer and promote a strengths based approach.

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The listener should invite the person/carer to:

- Talk about a time or times when something has been working well in their life;
- Explore what it was that worked well and supported them at that time;
- Think about how that experience could support them now in making a plan for the future.

Some key questions to support an appreciative enquiry approach include:

- Tell me about a time when things were going well for you?

- b. What did you learn about your strengths at that time?
- c. If you had a magic wand what would the future look like?
- d. What is it that you value most in your life now?
- e. What small changes would make the most difference?

The use of tools

A tool can be helpful to shape and focus a conversation about need, making sure that you consider everything that needs to be considered from a statutory perspective. Tools are also useful because they:

- a. Can be completed with the person/carer as part of any conversation you have with them; or
- b. The person/carer can complete them in their own time as part of their preparation for the conversation.

See: [Tools and Practice Guidance to Establish Needs](#) to access the range of tools that are available and practice guidance around the use of tools.

6.6 Fluctuating need

To ensure an accurate assessment of need it is important that you identify whether a person/carer has needs that fluctuate (change over time).

Fluctuating needs are needs that:

- a. May not be apparent at the time of the assessment; but
- b. Have been an issue in the past; and
- c. Are likely to arise again in the future.

A simple question to ask a person/carer to establish whether their needs fluctuate is 'How do your needs change over time?'

Every person/carer with fluctuating needs will experience fluctuations differently. For example:

- a. Some people's needs fluctuate throughout each and every day;
- b. Some people's needs fluctuate on some days each week;
- c. Some people will have periods of stability that could last several weeks or months with no fluctuation;
- d. Some people's fluctuations are related to a physical or mental health condition (for example Parkinson's Disease or Bi-Polar Disorder); and
- e. Some fluctuations are a response to a change in circumstance or environment (for example the weather affecting arthritis).

Because of the variance in fluctuation possible it is not appropriate to simply assess a person/carer as having 'fluctuating' needs. Neither should you make a judgement about need based solely on either a 'worse case' or a 'best case' scenario. Doing so would not provide a true representation of the person's/carer's whole needs and it would be likely that any support the Local Authority did subsequently provide would be inappropriate or disproportionate (too much or not enough) to cover periods of fluctuation.

You need to be able to effectively demonstrate:

- a. How often the person/carer experiences a fluctuation in needs;
- b. The type of fluctuations that are experienced and the impact on other needs and Wellbeing;
- c. What the potential causes or triggers are for fluctuations; and
- d. What measures are effective in managing periods of fluctuation or reducing the risk that they will occur.

The kind of questions you could ask a person/carer to help understand their fluctuating needs includes:

- a. How long has it been since you had a good/bad day?
- b. What does a good/bad day look like?
- c. What changes in support do you have on a bad day?
- d. How often do your needs change on a bad day?
- e. Which areas change the most on a bad day?

Need to know

Remember that using a tool to support the process of establishing needs or formal assessment can be useful for all involved.

The length of extension to an assessment process to ensure that fluctuating needs are understood should be based on the individual needs and circumstances of the person/carer and could be a day, a week or several months in duration.

It may also be appropriate to pause an assessment to enable the person/carer to access support that may then prevent or reduce the number of fluctuations occurring (for example to allow a new medication to take effect or to access to a health service to support anxiety).

6.7 Talking about need when the person lacks Capacity

Some people with Care and Support needs will lack capacity to have a conversation or communicate information about their needs (verbally or through another means). For example, they may be too unwell to do so or have a significant learning disability. Where this is the case the duty to ensure their involvement still applies.

There are a range of ways that you can ensure the involvement of a person who lacks capacity, including but not limited to:

- a. An appropriate other person or independent advocate to support the person to engage and ensure that they are represented;
- b. Spending time with the person can show you what needs they have and how they are currently being supported;
- c. Consulting with a range of people who know the person before reaching a judgement about need. Speaking to a family member, a health professional, a paid carer, a college tutor and a day services manager will give a much better picture of the person's needs than relying on the views of one person;
- d. Use other available evidence to support you to understand need (for example ABC charts, carer's notes and other records).

All information gathering and sharing should be carried out with regard to the Caldicott Principles, Data Protection legislation and local information sharing policies.

6.8 Exploring and Managing different views about need

Different people involved with the person/carer are likely to have different views about need. For example, the person/carer them self is likely to have a subjective view about their needs, which may be dominated by worries or concerns they have about their presenting situation (for example the potential impact of a serious health diagnosis), whereas someone who is emotionally removed from the situation may see things more objectively.

It is important that you recognise when a person's/carer's views about their needs may not be holistic and take steps to try and support them to gain insight or broaden their thinking. One way of doing this is to seek the views of family members and others involved in their life (with the person's/carer's consent or in their best interests if they are a person with Care and Support needs and lack capacity). This supports a whole family approach to assessment but can also lead to a shared understanding and solution from within informal networks of support.

Even with the involvement and perspective of others certain needs may still be difficult for people to recognise, acknowledge or explore. If it is your view that these needs exist and are having an impact on the person/carer then you must raise them for the purposes of:

- a. Supporting the person/carer to gain insight into their needs and situation;
- b. Developing the person's/carer's resilience through talking about difficult situations and exploring options to find a solution;
- c. Providing information and advice;
- d. Preventing, reducing or delaying the need for Care and Support/Support.

Example:

Peter has Autism and lives in his own tenancy within a supported living scheme. When asked to describe his needs around meal preparation he says that he has no needs. However, the social care practitioner is aware that Peter's parents support him to prepare his main meals for the week every Sunday and feel that he lacks some insight and understanding around his needs. The social care practitioner asks Peter to break down the stages of meal preparation which supports him to recognise that there are some elements that he can do independently (using the microwave) but others where he has a need for support (cooking and shopping for food).

Example:

Maureen tells her social care practitioner that she is no longer able to clean her home because of changes in her mobility. The social care practitioner knows that Maureen was carrying out some tasks until a few weeks ago when she fell over. Maureen recovered well from her fall but appears to have lost confidence in her abilities. She speaks to Maureen about this possibility and by doing so supports her to recognise that anxiety about falling is the barrier to her carrying out household tasks. Acknowledging this allows Maureen to think about ways that she can gain confidence to carry out the tasks once more.

You may also find the tool [Six Tips to Help You Have Difficult Conversations](#) useful.

You should also make effective use of supervision to explore and develop skills that will support you to have difficult and sensitive conversations with people in a positive way.

6.9 Talking about short term needs

Sometimes it is necessary to talk about needs that you know are likely to change at some point in the near future. For example:

- a. When a person/carer is expected to make a full recovery from a health related condition;
- b. When a person's/carer's needs are likely to be temporary (e.g. needs relating to upcoming surgery to resolve a physical need); or
- c. When a person's/carer's needs are related to their circumstances and these circumstances are going to change (e.g. needs relating to caring for a child when the child is going to be staying long term with another family member).

If those needs are eligible the duty to meet the needs applies, even if the need is only likely to be short term.

Where needs are likely to be changing over time it is important to make arrangements with the person/carer to monitor and review the needs over an appropriate timeframe that reflects the individual circumstances of the person/carer.

6.10 Talking about needs in a crisis

Sometimes it is necessary to establish needs during a period of crisis. For example:

- a. When an informal caring relationship has completely broken down with immediate effect; or
- b. When the person's/carer's physical or mental health condition deteriorates rapidly and current services being provided are no longer working.

At times of crisis the presenting needs that a person/carer has may not be a true representation of their on-going needs for care and support. As such it is important to make arrangements with the person/carer (or their representative) to monitor and review their needs and situation, re-assessing where there is evidence that long term needs are clear.

6.11 Talking about how to meet needs (Social Work)

At this point in the assessment process you will not yet have made a formal determination about eligibility, although you may have gathered enough information to give a good indication. As the formal determination has not yet been made you should restrict conversations about ways that needs could be met to:

- a. Ways that the person/carer could meet their needs (either independently or with the support of their informal support networks or community);
- b. Information and advice about prevention services that could delay, reduce or prevent needs; and
- c. Broad information about the type of services available in the marketplace that may be appropriate to meet needs (if the person/carer appears to have eligible needs).

A more specific conversation about how the Local Authority could meet needs will take place as part of the Care and Support/Support Planning process after:

- a. The formal eligibility determination is made; and
- b. The person/carer has confirmed that they wish for the Local Authority to meet their needs.

6.12 Talking about how to meet needs (Occupational Therapy)

Meeting needs with Equipment

If a decision is made that the provision of equipment is the best way to meet the identified need you should:

- a. Broadly explore the range of equipment that may be appropriate to meet needs;
- b. Explain that the provision of equipment by the Local Authority is subject to an eligibility determination under the Care Act;
- c. Provide information and advice about how appropriate equipment can be provided through other means (including how the person could access the equipment informally).

Meeting needs with an adaptation

When deciding whether a major adaptation is the best way to meet needs you should refer to:

- a. Local policy about adaptations that can be provided under the Housing Grants, Construction and Regeneration Act 1996 (HGCRA);
- b. Local policy about adaptations that cannot be provided under the HGCRA; and
- c. The needs domains as set out under section 23 of the Housing Grants, Construction and Regeneration Act 1996 (HGCRA) to make sure that the needs identified during assessment are likely to be eligible under the HGCRA.

If the person does not appear to have a need under the criteria used by the HGCRA a major adaptation may not be the most appropriate way to meet needs and alternative options may need to be explored.

If the person does appear to have a need under the HGCRA criteria, you will need to:

- a. Agree the nature of the adaptation required (for example 'the provision of a wet room' or 'extension of a kitchen');
- b. Provide information and advice about the available ways that the adaptation can be provided (including ways that the person can make their own arrangements);

- c. Explain that, if the person wishes for the Local Authority to arrange and carry out the adaptation this is subject to approval by the Housing Authority;
- d. Explain that the eligibility criteria of the HGCRA is applied to all major adaptation applications, and this will determine whether the works are carried out and whether a Disabled Facilities Grant is awarded;
- e. Explain that Disabled Facilities are subject to application and means testing.

When deciding the nature of the adaptation you must consider:

- a. All of the available ways to meet the identified need; and
- b. Whether there are other ways to meet the needs that are more proportionate.

If Occupational Therapy is not appropriate

If the Occupational Therapy service is not an appropriate way to meet needs (or if another service is required alongside Occupational Therapy) you must be able to provide information and advice about:

- a. Ways that the person could meet needs (either independently or with the support of their informal support networks or community);
- b. Information and advice about other prevention services that could delay, reduce or prevent needs;
- c. Broad information about the type of other services available in the marketplace that may be appropriate to meet needs (if the person appears to have eligible needs).

If the person is likely to need support to access an appropriate service you must take steps to facilitate this.

7. Talking about Outcomes (Assessment)

This section of the procedures should only be used when talking about outcomes as part of the process to establish needs. When talking about outcomes at any other time you should refer to the relevant procedure for the process taking place.

Need to know

Remember that using a tool to support the process of establishing needs or formal assessment can be useful for all involved.

During the assessment conversation about Wellbeing the person/carer should naturally identify:

- a. Things they want to achieve;
- b. Things they want to change; and
- c. Things they want to stay the same.

These things are known as outcomes. In order to promote Wellbeing you should explore with the person/carer during the assessment the steps that they can take to achieve their outcomes, as the two things are intrinsically linked.

If, following assessment the Local Authority meets any eligible needs that the person/carer has you will need to explore not only what steps the person/carer can take to achieve their outcomes, but also how the support/services provided can help them do so too.

Need to know

Outcomes should reflect the things that the person/carer wants to achieve and not what other people want to happen.

8. Talking about Risk

8.1 Understanding what risk is

Risk is broadly defined as 'the probability that an event will occur with beneficial or harmful consequences'.

The aim of any conversation about risk is to maximise the benefits and reduce the likelihood of harm.

8.2 Why talking about risk is important

When you have a conversation about promoting Wellbeing, meeting needs or ways to achieve an outcome there will at some point be a need to talk about risk and doing so at an early stage can support the person/carer to:

- a. Explore and understand the benefits of taking the risk;
- b. Explore and understand the potential harmful consequences of taking the risk;
- c. Think about the measures they can take to reduce the likelihood of a negative consequence; and
- d. Make an informed decision about whether to take the risk.

The process of talking about risk can be very empowering for a person/carer and build resilience, confidence and independence.

8.3 Talking about risk

Examples of risk you may need to talk about when establishing need can include:

- a. Risk associated with an impairment or disability (e.g. falls);
- b. Risk of accidental injury (e.g. from traffic when in the community or using a household appliance at home);
- c. Risks around a person's ability to manage medication;
- d. Risks around the use of drugs or alcohol; and
- e. Risk of abuse or neglect (e.g. exploitation by others);

Depending on the level of risk a formal risk assessment may need to be carried out in addition to any other assessment process.

See: [Risk Assessment](#).

9. Good Information and Advice and Prevention

The duty to provide good information and advice and to consider ways to prevent, reduce or delay needs for Care and Support applies at all times.

It is vital that you understand your duties in relation to the above. Please use the links below to access further information as required.

- [Preventing Needs for Care and Support](#) for information about the duty to prevent, reduce or delay needs;
- [Providing Information and Advice](#) for information about the duty to provide good information and advice, including the duty to make sure that information and advice is accessible to the person receiving it.

Before any conversation to establish needs begins it is important that the person/carer and everyone else involved understands:

- a. Your role and how you can be contacted;
- b. The role of anyone else involved (for example an advocate);
- c. Why the process is taking place;
- d. What is going to happen during the process, including how long it may take;
- e. What the possible outcomes of the process may be and the implications (for example for the person's/carer's finances); and
- f. What is going to happen next, including how long it is likely to be until further contact is made and any further processes that may then be carried out (for example Care and Support/Support planning).

During the process it is important that you:

- a. Offer information and advice as appropriate, including signposting to other sources;
- b. Discuss options to prevent, reduce or delay the need for Care and Support; and
- c. Give the person/carer and anyone else involved opportunities to ask questions or seek clarity.

Depending on the person's/carer's situation and needs specialist information and advice that may be required could include:

- a. Advice around the financial assessment process;

- b. Advice about becoming a Lasting Power of Attorney or Deputy;
- c. Advice about becoming an Appointee;
- d. Advice about making a complaint about the Local Authority;
- e. Information about the local market place for Care and Support or health services.

See: [Providing Information and Advice](#), which includes access to local and national information and advice resources (general and specialist).

Need to know

If the carer works, always check that they are aware of their right to unpaid leave under the Carers Leave Act 2023. For further information, see: [Unpaid carers leave](#).

10. Safeguarding and Deprivation of Liberty

10.1 Safeguarding

If, as part of any conversation you have with a person/carers or their family you become concerned that a vulnerable adult or a child is experiencing, or at risk of abuse or neglect you **must** respond appropriately.

See [Safeguarding Adults](#), which also includes information about how to raise a concern about a person under the age of 18.

If you are concerned that an adult or child is in imminent danger from abuse or neglect, or that a criminal act has taken place you should contact the police by dialling 999.

Where the safeguarding is in respect of the person/carers whose needs are being established a decision will need to be made about the need to pause the assessment process to allow a safeguarding enquiry to take place.

There are 3 possible options:

- a. The assessment process continues alongside any safeguarding process;

- b. The assessment is paused with no on-going intervention by the person carrying out the assessment whilst a safeguarding process takes place;
or
- c. The assessment is paused but urgent interim support is arranged to ensure needs are met whilst a safeguarding process takes place.

Any decision should involve the person carrying out the needs assessment, the person who will be carrying out any safeguarding process, the person/carer being assessed and, in the case of an assessment for a person with Care and Support needs, any carer.

10.2 Deprivation of Liberty

You must consider any appropriate action required to authorise deprivations of liberty whenever:

- a. The person with Care and Support needs lacks capacity to make decisions about the Care and Support provided to them; and
- b. You feel the level of restriction being imposed on the person is depriving them of their liberty; or
- c. You feel the level of restriction required to meet their care and support needs following assessment is likely to deprive them of their liberty.

See: [Recognising and Responding to Deprivations of Liberty](#).

11. Pausing the Conversation

Under the Care Act an assessment should be carried out in a timely way based upon the needs and circumstances of the person/carer.

Sometimes it may be appropriate to pause the process part way through. For example:

- a. Safeguarding concerns have been raised and a safeguarding process is to take place;
- b. To allow a service aimed at delaying, preventing or reducing needs for Care and Support to be provided (for example a period of reablement or a piece of equipment);
- c. To allow the person/carer time to recuperate following a period of ill health;
- d. In response to a change in the person's/carer's personal circumstances (for example a death or loss of work).

Any decision to pause the process should be made with regard to:

- a. The person's/carer's thoughts, wishes and feelings;
- b. The likely impact on the person's/carer's Wellbeing; and
- c. The views of any other person involved or being consulted.

Whenever the conversation recommences you must make sure that you review the original information gathered and ensure that any changes in need and Wellbeing that have occurred are reflected.

12. Making Notes

It is quite appropriate to take notes during a skilled conversation to ensure that you are able to satisfactorily recall and capture what has been discussed and agreed in a formal record. However, there are some general good practice rules to follow when doing so:

- a. Think beforehand about the level of note-taking that may be required-ensure you have to right tools and that they are proportionate;
- b. Explain to the person/carer and anyone else present that you will be taking some notes and why;
- c. Reassure the person/carer and anyone else present that you will still be listening to them even when you are making notes;
- d. Don't record everything that is said. This will prevent you from engaging in the conversation and cause distraction-you need to pick out what is relevant and important;
- e. Make sure you record everything that the person/carer says is important to them, even if it does not appear to be relevant to you or others;
- f. If the person/carer uses a certain phrase that is powerful or indicative to the context you should record this word for word;
- g. Sometimes information is detailed or complex and taking notes could take a little longer than expected. If this is the case you should consider making a polite request for a brief pause to allow for notes to be made;
- h. Sometimes people provide a lot of information without a pause-perhaps they are anxious or simply have a lot to say. Trying to keep up can lead to you over-recording (recording everything regardless of relevance); missing key points, failing to understand what is being said or appearing disengaged from the conversation. If this is the case you should consider politely requesting a pause to allow for clarity and notes to be made;

- i. Refer to your notes to summarise what has been said during the conversation, reflect and seek clarity about what has been agreed and next steps;
- j. Try to make notes in a legible way and take care to use appropriate language - the person/carer may request to see the notes or be provided with a copy;
- k. Confidentiality must be maintained at all times. Make sure that the notes are kept securely and only available to people authorised to see them;
- l. Always file or dispose of any notes securely when a formal record of the conversation has been made.

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